

PhloemAI Interview Question Mark Scheme Rubrics
20 worked structure examples for medical interview practice

How to use this document:

Use the structure to plan a concise spoken answer. Use the mark scheme points after answering to decide whether the response is complete enough, balanced enough, and linked back to medicine.

Question 01: What has influenced your decision to pursue a career in medicine?

Category: Personal and Motivation

Structure:

Start: Give a direct reason, then name the experience or value that made it real.

Middle: Explain one or two specific experiences and what they taught you about medicine.

End: Link the reflection to patient care, long-term commitment and realistic preparation.

Mark Scheme:

General:

- Motivation sounds personal, specific and reflective rather than generic.
- Answer shows realistic understanding of the demands of medicine.
- Patient care, service or responsibility is clearly present.

Start:

- Opens with a clear answer to why medicine.
- Gives one personal influence rather than a list of vague ideas.

Middle:

- Uses evidence from work experience, volunteering, reading or personal reflection.
- Explains what changed in your understanding after that experience.

End:

- Concludes with why medicine remains a considered choice.
- Avoids exaggeration and shows readiness to keep learning.

Question 02: In what ways have you explored whether medicine is genuinely the right career for you?

Category: Personal and Motivation

Structure:

Start: State that you tested the decision actively rather than assuming it.

Middle: Compare different sources of insight, such as clinical exposure, volunteering and speaking to professionals.

End: Explain what confirmed the fit and what challenges you still recognise.

Mark Scheme:

General:

- Shows active exploration, not passive interest.
- Includes both positives and difficulties of the career.
- Reflects on personal suitability.

Start:

- Directly answers how you explored the choice.
- Makes clear that your decision was tested over time.

Middle:

- Gives specific examples and lessons learned from each.
- Mentions the realities of pressure, teamwork, uncertainty or responsibility.

End:

- Links exploration to a confident but realistic decision.
- Shows humility about future learning.

Question 03: What lessons did you take from your work experience or clinical exposure?

Category: Personal and Motivation

Structure:

Start: Pick one main lesson rather than summarising every placement.

Middle: Describe the moment briefly, then analyse why it mattered.

End: Connect the lesson to the kind of medical student or doctor you want to become.

Mark Scheme:

General:

- Reflection is deeper than description.
- The example is specific enough to feel credible.

- The answer links learning to future behaviour.

Start:

- Names the main lesson clearly.
- Sets context without overlong storytelling.

Middle:

- Explains how the lesson affected your view of patients, staff or teamwork.
- Recognises limits of observation and avoids pretending to know everything.

End:

- States how you would apply the lesson in training.
- Finishes with a patient-centred link.

Question 04: What do you see as the biggest advantages and disadvantages of being a doctor?

Category: NHS and Healthcare

Structure:

Start: Give one advantage and one disadvantage upfront.

Middle: Develop both sides with realistic detail and examples.

End: Explain why the advantages still align with your values despite the challenges.

Mark Scheme:

General:

- Answer is balanced rather than idealised.
- Shows understanding of pressure, responsibility and uncertainty.
- Explains why the role is still attractive.

Start:

- Opens with a clear balanced frame.
- Avoids claiming medicine is simply rewarding or secure.

Middle:

- Discusses meaningful patient impact and intellectual challenge.
- Discusses workload, emotional pressure, mistakes or system strain.

End:

- Brings the answer back to motivation and preparedness.
- Shows maturity without sounding negative.

Question 05: How would you explain what patient-centred care means?

Category: NHS and Healthcare

Structure:

Start: Define patient-centred care in simple language.

Middle: Explain shared decision-making, listening and individual circumstances.

End: Link it to safety, trust and better outcomes.

Mark Scheme:

General:

- Definition is clear and not just a slogan.
- Patient autonomy and context are included.
- The answer links communication to quality of care.

Start:

- Gives a concise definition.
- Makes clear that the patient is an active partner.

Middle:

- Mentions listening to concerns, values and preferences.
- Explains that care should be clinically appropriate, not just whatever the patient asks for.

End:

- Connects patient-centred care to trust and adherence.
- Finishes with safe, respectful care.

Question 06: Why do you think empathy is important in medicine?

Category: Communication and Teamwork

Structure:

Start: Define empathy as understanding and responding to another person's experience.

Middle: Explain how empathy improves communication, trust and clinical information gathering.

End: Balance empathy with professionalism and boundaries.

Mark Scheme:

General:

- Empathy is linked to action, not just feeling sorry for someone.
- The answer connects empathy to patient care and safety.
- Boundaries are recognised.

Start:

- Gives a practical definition of empathy.
- Answers why it matters in medicine specifically.

Middle:

- Explains impact on trust, disclosure and shared decisions.
- Uses a patient example or clinical context.

End:

- Notes that empathy must be balanced with clear judgement.
- Ends with compassionate professionalism.

Question 07: How might you communicate with a patient who was frightened?

Category: Communication and Teamwork

Structure:

Start: Prioritise calmness, safety and understanding what they fear.

Middle: Use listening, simple explanations, checking understanding and support.

End: Summarise next steps and ensure they are not left unsupported.

Mark Scheme:

General:

- Response is empathetic and practical.
- Communication is adapted to the patient's emotional state.
- Safety and escalation are considered where needed.

Start:

- Acknowledges fear directly and respectfully.
- Avoids rushing straight into information-giving.

Middle:

- Uses open questions, pauses and plain language.
- Checks understanding and offers support from colleagues or family if appropriate.

End:

- Gives a clear next step.
- Confirms the patient knows how to ask for help.

Question 08: Describe a time when you contributed successfully to a team.

Category: Communication and Teamwork

Structure:

Start: Briefly set the task, team and your role.

Middle: Explain the action you took and how it helped the team.

End: Reflect on what you learned about teamwork in healthcare.

Mark Scheme:

General:

- Example is specific and credible.
- Your contribution is clear without overstating your role.
- Reflection links to future teamwork.

Start:

- Sets context quickly using a clear situation.
- States your role and the team's aim.

Middle:

- Shows communication, reliability or support for others.
- Explains how the outcome improved because of the team.

End:

- Identifies one lesson you would carry into medicine.
- Avoids claiming all success was due to you.

Question 09: How might you manage disagreement within a team?

Category: Communication and Teamwork

Structure:

Start: State that disagreement should be handled calmly and constructively.

Middle: Gather views, find shared goals, use evidence and involve a leader if needed.

End: Link the approach to patient safety and maintaining team trust.

Mark Scheme:

General:

- Avoids blame and focuses on resolution.
- Recognises that disagreement can be useful if managed well.
- Escalation is appropriate, not automatic.

Start:

- Frames disagreement as a communication and safety issue.
- Sets a professional tone.

Middle:

- Listens to each viewpoint and clarifies facts.
- Refocuses the group on shared priorities and evidence.

End:

- States when escalation would be needed.
- Ends with teamwork and safe care.

Question 10: How would you recognise when you are becoming overwhelmed?

Category: Personal and Motivation

Structure:

Start: Acknowledge that recognising pressure early is important.

Middle: Describe personal warning signs and practical responses.

End: Explain how asking for help protects patients and performance.

Mark Scheme:

General:

- Shows self-awareness without sounding unsafe.
- Gives practical coping strategies.
- Normalises seeking support appropriately.

Start:

- Directly answers how you would recognise it.
- Avoids pretending you never feel pressure.

Middle:

- Mentions signs such as reduced concentration, irritability or poor organisation.
- Uses strategies such as prioritising, rest, speaking to tutors or peers.

End:

- Links wellbeing to professional responsibility.
- Shows resilience as adaptation, not silent endurance.

Question 11: Which challenges currently face the NHS?

Category: NHS and Healthcare

Structure:

Start: Name two or three major challenges rather than listing everything.

Middle: Explain causes and impact on patients and staff.

End: Give a balanced view of improvement, teamwork and realistic constraints.

Mark Scheme:

General:

- Shows accurate broad awareness of the NHS.
- Balances patient, staff and system perspectives.
- Avoids unsupported or party-political claims.

Start:

- Identifies clear issues such as waiting times, workforce pressure or demand.
- States why the issue matters.

Middle:

- Explains how challenges interact rather than treating them separately.
- Gives practical examples of impact on care.

End:

- Suggests realistic improvement areas.
- Links back to the responsibilities of future doctors.

Question 12: How could the NHS decide which services to prioritise?

Category: NHS and Healthcare

Structure:

Start: State that prioritisation should be fair, evidence-based and transparent.

Middle: Weigh clinical need, effectiveness, cost, equity and public input.

End: Explain that no system is perfect, so decisions must be reviewed and justified.

Mark Scheme:

General:

- Recognises scarcity and fairness.
- Uses criteria rather than personal preference.
- Considers both individual and population-level benefit.

Start:

- Defines the decision-making problem.
- Avoids saying first come, first served without nuance.

Middle:

- Discusses urgency, benefit, cost-effectiveness and inequalities.
- Acknowledges independent bodies, clinicians and patient voices.

End:

- Gives a justified balanced conclusion.
- Mentions transparency and review.

Question 13: How would you define informed consent?

Category: Ethics and Professionalism

Structure:

Start: Define consent as voluntary agreement after receiving and understanding relevant information.

Middle: Explain capacity, information, risks, alternatives and the right to refuse.

End: Link consent to autonomy, trust and legal/professional responsibility.

Mark Scheme:

General:

- Definition includes information, understanding, capacity and voluntariness.
- The patient is treated as an active decision-maker.
- Risks and alternatives are included.

Start:

- Gives a precise definition.
- Mentions that consent is a process, not just a signature.

Middle:

- Explains capacity and freedom from pressure.
- Includes benefits, risks, alternatives and doing nothing.

End:

- Links to respect for autonomy and trust.
- Mentions checking understanding.

Question 14: In what situations might confidentiality need to be breached?

Category: Ethics and Professionalism

Structure:

Start: State that confidentiality is central but not absolute.

Middle: Explain serious risk to the patient, others, safeguarding or legal requirements.

End: Say disclosure should be minimal, justified and ideally discussed with the patient.

Mark Scheme:

General:

- Shows respect for confidentiality.
- Recognises clear exceptions based on risk or law.
- Proportionate disclosure is included.

Start:

- Avoids saying confidentiality can never be broken.
- Frames breach as exceptional and justified.

Middle:

- Mentions safeguarding, serious harm, communicable disease or court/legal duties.
- Explains seeking senior advice where appropriate.

End:

- States disclose only necessary information to appropriate people.
- Mentions documenting reasoning.

Question 15: What do you see as the four main principles of medical ethics?

Category: Ethics and Professionalism

Structure:

Start: Name autonomy, beneficence, non-maleficence and justice.

Middle: Define each principle with a brief healthcare example.

End: Explain that principles can conflict and must be balanced.

Mark Scheme:

General:

- All four principles are named accurately.
- Definitions are applied rather than memorised.
- Conflict between principles is recognised.

Start:

- Lists the four principles clearly.
- Avoids spending too long before answering.

Middle:

- Gives simple definitions and relevant examples.
- Shows how principles guide but do not automatically decide every case.

End:

- Notes the need for balanced judgement.
- Links ethical reasoning to patient-centred care.

Question 16: A competent adult refuses a blood transfusion for religious reasons even though refusing it could result in death. How might you approach the situation?

Category: Ethics and Professionalism

Structure:

Start: Identify autonomy, capacity, information and seriousness of harm.

Middle: Confirm capacity, explore understanding, discuss alternatives and avoid coercion.

End: Respect a competent refusal, document carefully and involve senior support.

Mark Scheme:

General:

- Recognises a competent adult can refuse treatment.
- Balances autonomy with duty to explain risk and offer alternatives.
- Communication remains respectful of beliefs.

Start:

- States the ethical tension clearly.
- Does not jump to forcing treatment.

Middle:

- Assesses capacity and understanding of consequences.
- Explores reasons, alternatives and specialist input.

End:

- Respects the decision if capacity is present.
- Documents, escalates and supports ongoing care.

Question 17: Why do you think medical research is important?

Category: Data, Research and Critical Thinking

Structure:

Start: State that research improves evidence, treatments and patient outcomes.

Middle: Explain examples such as new treatments, prevention, safety and service improvement.

End: Balance innovation with ethics, consent and avoiding overclaiming.

Mark Scheme:

General:

- Research is linked to patient benefit.
- Evidence and uncertainty are understood.
- Ethics and safety are mentioned.

Start:

- Gives a direct reason for importance.
- Avoids vague praise for science only.

Middle:

- Discusses how research changes practice and tests assumptions.
- Mentions quality, safety or population health.

End:

- Notes that research must be ethical and critically appraised.
- Finishes with better care as the purpose.

Question 18: How would you define evidence-based medicine?

Category: Data, Research and Critical Thinking

Structure:

Start: Define it as using best available evidence alongside clinical judgement and patient values.

Middle: Explain guidelines, research appraisal, limitations and individual context.

End: Link to safer decisions and adapting when evidence changes.

Mark Scheme:

General:

- Definition includes evidence, clinician judgement and patient preference.
- Shows evidence is interpreted, not followed blindly.
- Limitations and uncertainty are included.

Start:

- Gives a concise definition.
- Avoids saying it is only following guidelines.

Middle:

- Explains quality of evidence and patient-specific context.
- Mentions that evidence can evolve.

End:

- Links to safer, fairer and more effective care.
- Shows willingness to update practice.

Question 19: Which opportunities could artificial intelligence create in medicine?

Category: Hot Topics and Current Affairs

Structure:

Start: Name realistic opportunities such as triage, imaging support, administration or personalised care.

Middle: Explain benefits while balancing risks such as bias, data privacy and overreliance.

End: Conclude that AI should support, not replace, responsible clinical judgement.

Mark Scheme:

General:

- Shows current awareness without hype.
- Balances opportunity with safeguards.
- Patients and healthcare workers are both considered.

Start:

- Gives concrete examples of AI use.
- Avoids claiming AI will simply replace doctors.

Middle:

- Explains how AI could improve speed, access or consistency.
- Discusses bias, privacy, accountability and clinician oversight.

End:

- Gives a measured final view.
- Links back to safe, compassionate care.

Question 20: A patient becomes verbally aggressive towards you. How might you manage the situation?

Category: Practical MMI and Role Play

Structure:

Start: Stay calm, ensure immediate safety and acknowledge distress.

Middle: Listen, use de-escalation, set boundaries and seek support if risk increases.

End: Agree next steps, document if appropriate and protect staff and patient safety.

Mark Scheme:

General:

- Prioritises safety and de-escalation.
- Shows empathy without accepting abuse.
- Escalation is proportionate.

Start:

- Uses calm body language and tone.
- Acknowledges the emotion without arguing.

Middle:

- Explores the cause of anger and listens actively.
- Sets clear boundaries and involves staff/security if needed.

End:

- Offers a practical next step or follow-up.
- Documents and reflects if the incident affects care or safety.

